



Patient Profile

Name: Mr. PL94

Age: 35

Gender: Male

Summary of Results

The predictive model flags the overall laboratory profile as **Abnormal**. Elevated total and direct bilirubin levels, a mild rise in SGOT, and a low alkaline phosphatase value collectively suggest hepatic involvement warranting further clinical evaluation.

Detailed Test Explanations

Bilirubin Total – Measures overall bilirubin in blood. A value of 1.4 mg/dL exceeds the normal range (0.0-1.2 mg/dL), indicating hyperbilirubinemia.

Bilirubin Direct – Reflects conjugated bilirubin; 1.2 mg/dL is above its reference limit (0.0-0.2 mg/dL), pointing to a cholestatic or obstructive pattern.

Serum Bilirubin (Indirect) – Unconjugated bilirubin; 0.20 mg/dL falls within the normal 0.00-0.90 mg/dL range.

SGOT / AST – Liver transaminase reflecting hepatocellular injury; 36 U/L is slightly above the upper limit of 33 U/L, suggesting mild hepatocellular stress.

SGPT / ALT – Another hepatocellular marker; 35 U/L is within the normal 0-41 U/L range, implying limited enzyme elevation.

Alkaline Phosphatase (ALP) – Enzyme associated with bile ducts and bone; 25 U/L is below the lower bound of 40-129 U/L, which may reflect low bone turnover or a specific liver pattern.

Total Protein – 6.9 g/dL lies within 6.4-8.3 g/dL, indicating adequate protein synthesis.

Albumin – 4.6 g/dL is within 3.5-4.8 g/dL, showing normal hepatic protein production.

Globulin – 2.3 g/dL falls in the 1.9-3.7 g/dL range, normal.

Albumin/Globulin (A/G) Ratio – 2.0 g/dL is within 1.0-2.1, signifying balanced protein distribution.

Overall Interpretation

The profile exhibits a predominantly cholestatic pattern characterized by elevated direct bilirubin and a modest rise in SGOT, while SGPT remains normal. The low alkaline phosphatase is atypical for isolated cholestasis and may represent a concurrent bone or nutritional issue. These laboratory findings are not consistent with a purely normal hepatic function and thus the predictive model correctly flags them as abnormal.

Recommendations

- Reevaluate Liver Function** – Repeat the bilirubin panel, transaminases, and ALP within 4–6 weeks to assess trend.
- Imaging** – Consider abdominal ultrasound or MRCP to identify biliary obstruction or gallbladder pathology.
- Medication and Alcohol Review** – Document current drugs and alcohol intake; discontinue any hepatotoxic agents if possible.
- Dietary Assessment** – Ensure adequate intake of nutrients that support liver health (e.g., protein, vitamin D).

5. **Follow-up** – Schedule a hepatology consult if bilirubin or transaminase levels remain elevated or if symptoms such as jaundice, pruritus, or right upper quadrant pain develop.

Disclaimer

This report is for informational purposes only and does not substitute professional medical advice. Consult a qualified healthcare provider for diagnosis and treatment.